



Physiotherapy & Rehabilitation Platform

Medical Report: Chronic Lumbar Disc Herniation with Sciatica

Patient Information

Name: ahmet kalumian

Birth Date: 2025-10-28

Case ID: #22

Clinical Details

Doctor: Dr. Alarkany

Date: 2026-04-28

Description: Patient presents with persistent radiating pain from the lower back down to the right calf. Diagnosed with L4-L5 disc protrusion. Pain is aggravated by prolonged sitting and forward bending.

Physical Measurements

Height: 175 cm

Weight: 70 kg

Blood Group: AB+

Health History

Smoker: Yes

Chronic Diseases: Yes

Activity Level: Moderate

Smart Clinical Analysis

Clinical Explanation:

The patient presents with persistent radiating pain from the lower back to the right calf, attributed to an L4-L5 disc protrusion. The pain is exacerbated by prolonged sitting and forward bending, indicating a need for careful management of activities that may increase spinal loading. The patient's moderate activity level and chronic disease history necessitate a cautious approach to rehabilitation, focusing on pain relief, functional mobility, and gradual strengthening of the lumbar and lower extremity musculature.

Immediate Physiotherapy Recommendations:

A tailored physiotherapy program should include core stabilization exercises, gentle stretching, and modalities for pain relief such as heat or ultrasound. Emphasis should be placed on maintaining proper posture and ergonomics during daily activities. Gradual progression of exercises should be monitored closely to avoid exacerbation of symptoms.

Precautions:

The patient should avoid prolonged sitting and forward bending activities that may aggravate pain. High-impact activities and heavy lifting should also be avoided until significant improvement is observed. Close monitoring for any signs of increased neurological deficits is essential.

Estimated Recovery Timeline:

Given the nature of the condition and the patient's chronic disease status, a recovery timeline of 8 to 12 weeks is anticipated, with gradual improvement expected as rehabilitation progresses.

Risk Assessment:

Potential risks include worsening of disc protrusion symptoms, development of chronic pain syndrome, or neurological compromise if symptoms are not managed appropriately. Regular reassessment is crucial to mitigate these risks.

Red Flags:

Emergency warning signs include sudden onset of severe pain, loss of bowel or bladder control, progressive weakness in the lower extremities, or any new neurological symptoms. Immediate medical attention is required if these occur.

Test Group: Multiple Sclerosis

Test: Knee Flexion (Right)

Clinical AI Analysis

Doctor Standard/Target:
No doctor target was set

Clinical Interpretation:
The recorded knee flexion measurements show a progressive increase from 95 degrees to 115 degrees over the assessment period, indicating improvement in range of motion.

Progress Evaluation:
The patient has demonstrated a positive trend in knee flexion, with a total increase of 20 degrees, suggesting effective rehabilitation strategies are being implemented. Continued improvement is expected as therapy progresses.

Physiotherapy Focus:
Focus on maintaining and further improving knee flexion through targeted stretching and strengthening exercises for the lower extremity, while ensuring that exercises do not exacerbate lower back pain.

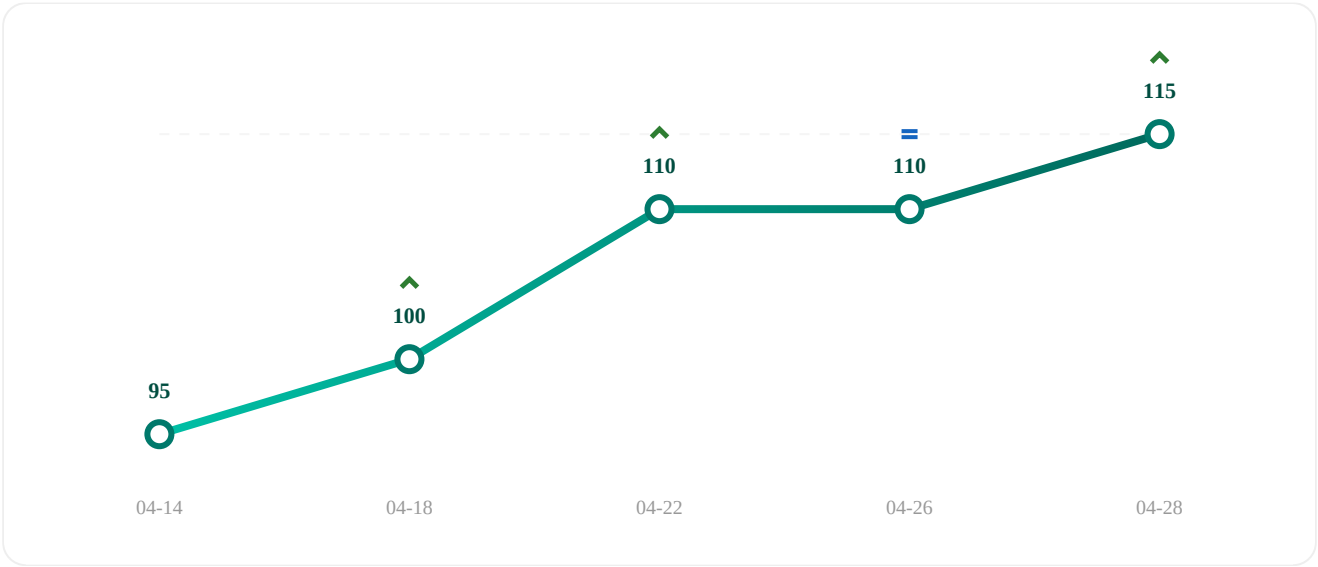
HIGHEST
115.0

LOWEST
95.0

AVERAGE
106.0

OVERALL PROGRESS

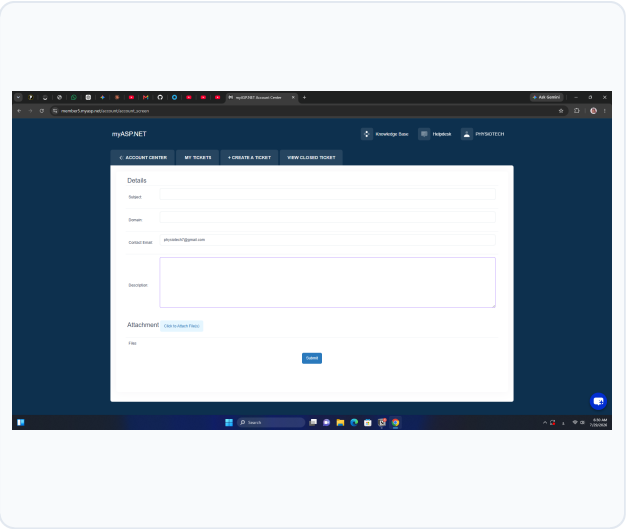
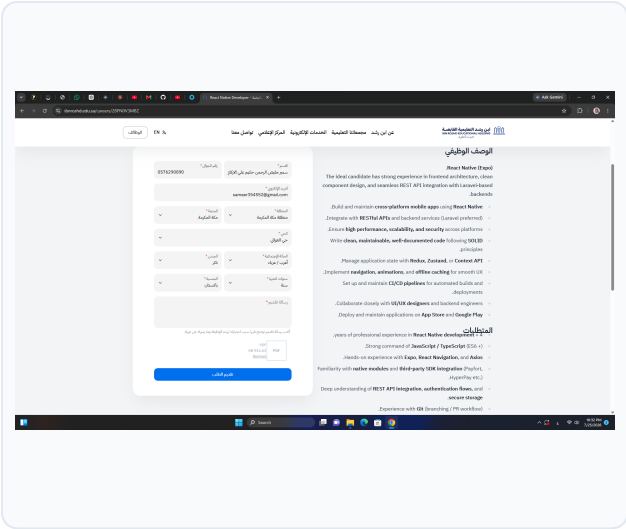
Increased by 21.1% ↑ Since first test (2026-04-14)



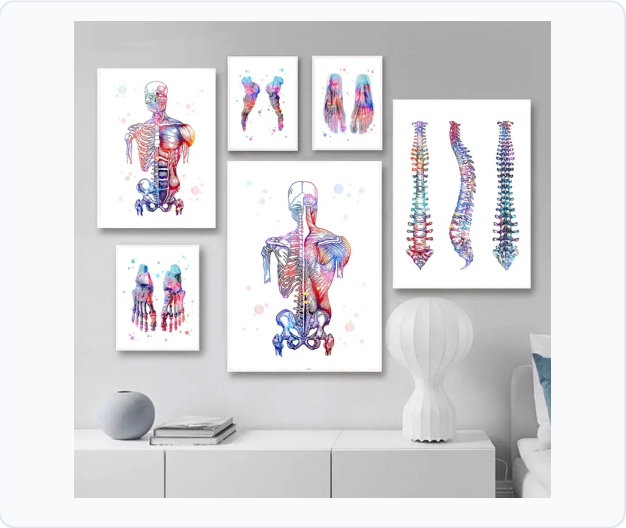
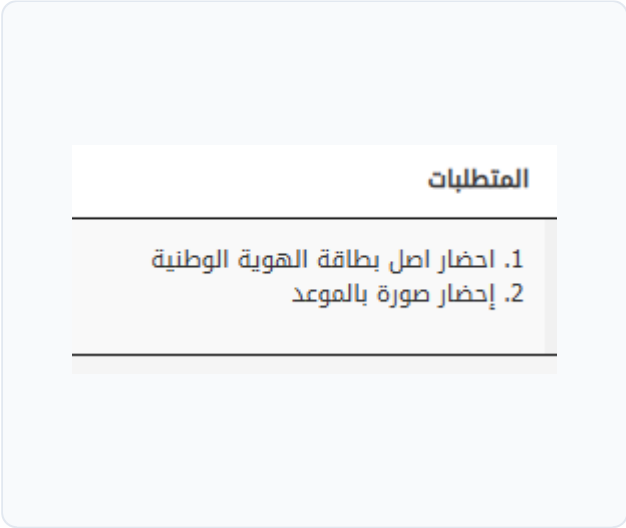
Date	Result	Standard
2026-04-14	95	—
2026-04-18	100	—
2026-04-22	110	—
2026-04-26	110	—
2026-04-28	115	—

Visual comparison: Knee Flexion (Right)

BEFORE



AFTER



Important Notice

This report is AI-assisted and intended for informational support only. It is not a medical diagnosis and does not replace professional clinical judgment. Please consult your doctor or therapist for any medical decisions.